

AML006 — Sarah Davis (DOB 11.11.1980)

Adm/Disch: 04.03.2024 → 11.03.2024

Dx: AML (ELN 2022: Favorable), CEBPA bZIP mut+

Tx line: 1L CPX-351 started 11.09.2023 (continues)

HPI: 43F with de novo AML, CEBPA bZIP, initially p/w fatigue, epistaxis, ANC 0.3, Plt 18, Hgb 7.6. Induced with CPX-351 (D1/3/5), achieved CR by 16.10.2023. This admit for consolidation #2 CPX-351 (D1/3). Tolerated well, expected cytopenias.

PMH: Mild persistent asthma, hypothyroidism (Hashimoto), migraine w/o aura, GAD.

PSH: C-section x2.

Allergies: NKDA.

FHx/Soc: Non-smoker, occ. wine, lives with spouse, 2 children. Works as HR manager.

Objective:

- **Vitals (on DC):** afeb, BP 112/68, HR 78, SpO₂ 99% RA.
- **CBC trend:**
 - 04.03: WBC 3.1 (ANC 1.2), Hgb 9.8, Plt 142
 - Nadir 08.03: WBC 0.4 (ANC 0.05), Hgb 8.7, Plt 38
 - 11.03: WBC 0.9 (ANC 0.18), Hgb 9.1, Plt 48
- **CMP (11.03):** Cr 0.8, eGFR >90, AST 24, ALT 19, TBili 0.7, LDH 182, uric acid 3.2.
- **Coag:** INR 1.0, fibrinogen 370.
- **Path/Flow:** Bone marrow 27.11.2023: cellularity 40%, blasts <2%, MRD by flow <0.1%, CEBPA bZIP persists as founder mut; no new mutations.
- **Imaging:** CXR nl. Echocardiogram baseline (09.2023): LVEF 62%. Port: tunneled R-IJ placed 09.2023, patent.

Hospital Course/Issues:

1. **CPX-351 consolidation #2** administered per protocol.
2. **FN prophylaxis:** levofloxacin 500 mg qd during ANC <0.5; posaconazole DR 300 mg qd; acyclovir 400 mg bid.
3. **TLS risk low;** maintained hydration, allopurinol d/c'd after stable urate.
4. **Cytopenias:** transfused 1U pRBC (10.03, Hgb 7.6), 1U platelets (08.03, Plt 9).
5. **Nausea** grade 1; ondansetron PRN helped.
6. **Asthma stable;** no steroids needed.
7. **Psych:** GAD controlled on sertraline; brief supportive counseling.

Assessment:

- AML (Favorable, CEBPA bZIP) in CR after CPX-351 induction; now s/p consolidation #2 with expected pancytopenia; no infectious complications; remains transfusion-dependent intermittently. Ongoing 1L therapy without evidence of relapse.

Plan/Dispo:

- **Home** with neutropenic precautions.
- **Meds on DC:**
 - Posaconazole 300 mg qd (while ANC <0.5)

- Levofloxacin 500 mg qd (while ANC <0.5)
- Acyclovir 400 mg bid (until ANC >1.0 x2 checks)
- Levothyroxine 75 mcg qAM
- Sertraline 50 mg qd
- Inhaled budesonide-formoterol 2 puffs bid; albuterol PRN
- Ondansetron 8 mg q8h PRN; senna 8.6 mg qHS PRN
- **Follow-up:** CBC 2–3×/wk at day hospital; transfuse to Hgb ≥8, Plt ≥10 (≥20 if bleeding).
- **Next steps:** Marrow MRD re-check planned ~April 2024 before consolidation #3; consider de-escalation to observation if sustained MRD negativity; no allo-HSCT planned given ELN-favorable risk and stable course.
- **When to call:** T ≥38.0°C, rigors, bleeding, dyspnea, uncontrolled N/V, catheter issues.

Sign: Hem/Onc service, 11.03.2024